جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : صبيره حنفي محمود اسماعيل
المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** ---
رقم البطاقة : 25003210102209 **رقم التليفون :** 01020017897 **تاريخ تسجيل الإستمارة :** 10-10-2026
العمر(تاريخ الميلاد) : 21-03-1950 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: صبيره حنفي محمود اسماعيل
National ID: 25003210102209

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Name : sabrya haufy mahmoud	Date : 06-01-2026 12:42
Branch : Portable	Medical No. : GRI-25003210102209
Ref.Physician Prof :	Ins.Company :

MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED :

Known case of left breast cancer under hormonal treatment, compared to the last CT report dated 22/6/2025, the current study revealed:

I- CHEST:

- The left breast showed heterogenous parenchymal area with nipple retraction and minimal skin thickening measures about 3.6mm.
- Prominent right axillary lymph node with few of them showed cortical thickening, the largest measures about 1 cm **for further ultrasound examination.**
- Still noted subcentimeteric mediastinal lymph nodes.
- No pulmonary nodules, masses or cavitary lesions.
- Bilateral middle lung lobes atelectatic bands are noted.
- No pleural or pericardial collection.
- Average cardiac size with atherosclerotic calcific foci of the aorta.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous fatty parenchymal attenuation.
- Both kidneys are normal site, relatively small in sized (measures about 8.8x4.1 cm on right side and 8.5x4.4 cm on left side) and showing smooth regular cortical outline and preserved parenchymal thickness. No stones or back pressure changes.
- Normal non-contrast C.T appearance of the spleen, pancreas, right suprarenal gland. Bulky left sided suprarenal gland.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Tiny calcific focus is seen within posterior uterine wall likely calcified myoma.
- No abdominal or pelvic sizable lymphadenopathy.

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Name : sabrya hanfy mahmoud	Date : 06-01-2026 12:42
Branch : Portable	Medical No. : GRI-250032101022O9
Ref.Physician Prof :	Ins.Company :

- Multiple pelvic calcific phleboli are noted.
- No ascites.
- Clear lung bases.
- **The examined skeleton** showed transpedicular fixation of L3 down to L5 associated with burst fracture of L4.

Opinion:

- **Stationary course since the last CT examination.**

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

CT Scan
[[DOCTYPE:CT_SCAN]]

NILE
SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Sabirah Hanafi Mahmoud	Date : 21-08-2025 11:08
Branch : Nasrcity	Medical No. : 179685

MULTI-SLICE ^{18}F-FDG PET/CT EXAMINATION OF THE BODY

Clinical History:

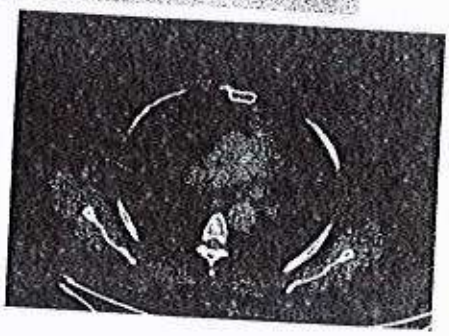
- 75 years old female with history of left breast cancer, underwent lumpectomy as well as adjuvant hormonal therapy.
- Follow-up study, compared to the previous PET/CT study report (No DICOM available) dated 14/1/2025.

Procedure:

- Total body PET scan one hour after IV injection of ^{18} F-FDG.
- MSCT performed and used for attenuation correction, anatomical localization and diagnosis.
- A semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 2.5.
- Mediastinal pool SUV_{max}~2.6 and hepatic SUV_{max}~4.

Findings:

❖ Chest:



- Left mastectomy clear operative bed showing no focal metabolically active lesions. No ipsilateral metabolically active axillary LNs could be detected.
- No metabolically active right breast lesions or axillary lymphadenopathy could be detected.
- Left lung anterolateral surfaces mild pleural thickening with underlying pulmonary radiating fibrotic bands, likely representing post-radiation changes.
- No FDG avid lung nodules, masses, consolidation or cavitary lesions detected.
- No metabolically active pericardial or pleural lesions detected.
- No mediastinal lymphadenopathy.
- Physiologic FDG uptake is noted in the heart, showing normal shape.
- Bilateral elastofibroma dorsi are noted showing low-grade metabolic activity.

❖ Head & Neck:

- Normal diffuse brain activity with no hypermetabolic brain lesions.
- Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx, showing normal appearance.
- Non-visualized thyroid gland? Surgically removed.
- No hypermetabolic cervical lymphadenopathy detected.

❖ Abdomen & Pelvis:

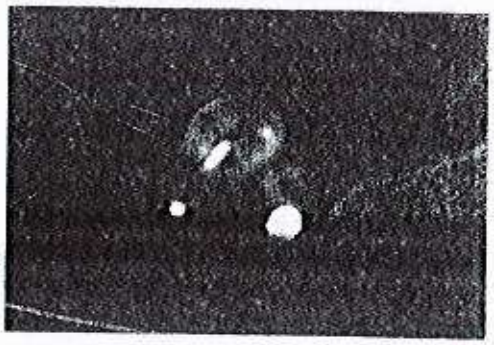
- Physiologic diffuse FDG uptake is seen over enlarged sized liver with diffusely low homogenous parenchymatous texture. No FDG avid focal lesions detected. No intra-hepatic or extra-hepatic biliary radicle dilatation.

PET Scan
[[DOCTYPE:PET_SCAN]]

NILE
SCAN & LABS

مركز النيل للاشعة والتحاليل

Name : Sabirah Hanafi Mahmoud	Date : 21-08-2025 11:08
Branch : Nasrcity	Medical No. : 179685



- Physiologic diffuse FDG uptake is seen over average sized spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes, physiologic FDG excretion is seen in the kidneys and bladder.
- Normal appearance of the pancreas, aorta, I.V.C and both adrenal glands.
- No metabolically active pelvic mass lesion seen.
- No hypermetabolic abdominal or pelvic lymphadenopathy.
- No FDG avid ascites or abdominal collections noted.

❖ **Musculoskeletal:**

- L3 down to S1 vertebral internal fixation with screws and plates, showing L4 vertebra plana with diffuse heterogenous metabolic activity, achieving SUVmax~6.2 (compared to SUVmax~3.7 in the previous study).
- Spondylo-degenerative changes of the spine.

OPINION:

Status: follow-up post-operative & therapeutic for a reported case of left breast cancer, showing:

- ***Clear left mastectomy operative bed showing no metabolically active loco-regional residual/recurrent lesions or lymphadenopathy.***
- ***Metabolic progressive course regarding the L4 vertebra plana, as described above.***

Thank you for referral,

Dr. Khaled Zaytoun
Radiologist

Dr. Omnia Talaat, MD
NCI - Cairo University

PET Scan
[DOCTYPE-PET_SCAN]

MDT form

A-Patient ID:
1- Name : Sex: Female
3- Medical number: 83164132
Old Med /
3570-2023
2-ID number:
Patient Age : 75 Y

Family history | Phone 01000964091 | BMI | REASON OF MDT
marital status | NRS Score | GROUP | Date of MDT 19/01/2025
Co morbidity | Menopausal status | Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	8/8	ve		10

T	N	M
2	3	1(INTER-PECT LN)

Sonomammography

PATIENT HISTORY

BIOPSY :poorly diff. adenocarcinoma
lt BCS T2N3M0
6 FAC , nolvadex ended 4 yrs ago
newly back pain L4 fraction , spinolamectomy , spinal fixation (no mets)
pt ct: interpectoral LN SVV 5.5
bone mets--> ibrance+femara-->PD
after revision of PET -CT : staionary course

MDT OUTCOME & PLAN

to continue on Ibrance / femara x 3 months then reassess
palliative RTH to L4

for Zometa / 3M

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

Patient Lastname: HANAFEY MAHMOUD
Patient Firstname: SABRA
HIS Patient ID: 10110225012848
RIS Patient ID: 373094
Date of Birth: 21/03/1950

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

========== REPORT TEXT ==========
99mTc MDP Bone Scan Report (3/8/2025)
* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

" No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
" Heterogonous tracer uptake is seen at lower lumbar vertebrae (known site of spinal fixation LV3-LV5) for MRI correlation.
" Degenerative changes are noted over the dorso-lumbar vertebrae.
" Arthritic changes are noted over both shoulder and both knee joints.
" Both kidneys are visualized.
Conclusion:
" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged
Areej Atran
Randa Ali, MSc

========== REPORT TEXT ENDE ==========

Patient Name: Sbirah Hanafy Mahmoud Ismail	Date : 22/06/2025
Exam : C.T. LUMBAR SPINE	Age : 75 Year(s)
Medical ID:	Pat. ID: 46868

MULTI-SLICE CT SCAN OF THE DORSO_LUMBAR SPINES

Findings:

Status post operative with transpedicular fixation of L3 ,L4 , L5 and S1 with plate and screws



- Burst fracture is seen involving the scanned L4 vertebral body with disrupted posterior elements, and retropulsed fragment is seen compromising the spinal canal upon destructive bony lesion involving L4 vertebral body and its posterior elements .
- Backward slippage of L4 over L5 due to fracture pars.
- Spondylodegenerative changes with marginal anterior osteophytes lipping & subchondral sclerosis with flowing anterior osteophytes.
- Normal diameters of the bony lumbar spinal canal.
- Facet arthropathy & normal both sacro-iliac articulations(Apart from osteoid osteoma of right iliac wing).
- Normal dimensions of dorsal spinal canal .

Conclusion:

- ***L4 destructive bony lesion likely metastatic with burst fracture.***
- ***Spondylodegenerative changes are seen involving the scanned vertebrae.***

Best regards

Dr. Reham Khedr..MD

Signature Dr.Mohamed Hassan

24/06/2025 1:06 AM

Signature Dr.Mohamed Hassan

24/06/2025 1:06 AM

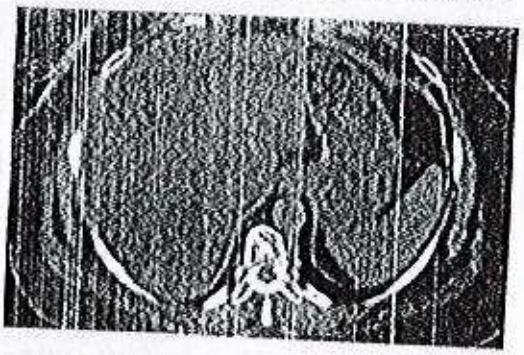
CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Sbirah Hanafy Mahmoud Ismail	Date : 22/06/2025
Exam : C.T CHEST	Age : 75 Year(s)
Medical ID: 3570/2023	Pat. ID: 46868

NON-CONTRAST CT SCAN OF THE CHEST , ABDOMEN AND PELVIS : -

Findings:

- Clear both lung fields
- **Few enlarged mediastinal lymph nodes , the largest seen at pretracheal space 3 x7mm ..**
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal appearance of the thoracic aorta and the great vessels.
- Average size liver showing fatty attenuation density . No C.T. evidence of hepatic focal lesions.



- * Average size spleen , no focal lesions.
- *Average size gall bladder , few tiny hyperdense gall stones.
- *No dilated intra or extra-hepatic bile ducts .
- *Patent main portal vein trunk and both branches.
- *Urinary bladder shows no masses or stones.
- *Normal C.T. appearance of the pancreas , adrenal glands, both kidneys ,bowel loops & other pelvic organs.
- *No retroperitoneal lymphadenopathy .
- *No ascites .
- **Lumbar spine fixation**

Opinion:

Fatty liver , otherwise unremarkable study.

DR/ Reham Khedr...MD

Signature Dr.Mohamed Hassan

24/06/2025 12:48 AM

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient No :25005210102200
Path No :SR-1103/23
Sex :Female
Age :73 Y
Referring Doc :

Invoice No :17093437
Request Date :05/10/2023
Verification Date :05/10/2023
Financial Category :
Department :Out Patient
Registered by :Ali mostafa

Pathology Report

Clinical Data: Left retropectoral mass, history of left breast cancer
Nature of Specimen: Slide Revision

Gross Examination:
- (1) paraffin block labeled by (780329) and (1) slides were received.

Microscopic Examination:
- Histological type: poorly differentiated adenocarcinoma
- IPT:
- ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 10% of nuclear reaction of tumor cells

Diagnosis:
Left retropectoral mass, Slide revision:
Poorly differentiated adenocarcinoma
Please correlate with the previous pathology.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 8/8)
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells

Signature
Prof. Dr Amal Hussein
Dr Heba Hassan

Verified by :
Dr Heba Hassan

Printing Date : 5/10/2023 11:41:35 Page 1 of 2 Printed By : Ali mostafa

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Dr.
Ella Anis Ishak
M.S., Ph.D. Pathology - Prof. of Pathology, Cairo University
Fellow, Royal College of Pathologists, U.K.
Member, United States and Canadian Academy of Pathology

Dr.
Shady Ella Anis
M.Sc. Hematology, Ain Shams University, Egypt
Fellow, Royal College of Pathologists, U.K.
Member, United States and Canadian Academy of Pathology

Patient Name : [illegible]
Analysis ID : [illegible]
Age : 73 Y 5 M 10 D
Gender : Female
Referred By Prof. Dr. : [illegible]
Clinical Diagnosis : Left retropectoral mass, history of left breast cancer
Nature of Specimen : US guided tru cut biopsy
Receiving Date : 2023-02-05
Patient ID : [illegible]

PATHOLOGY REPORT

Gross :

Multiple greyish white tissue cores, the longest measured 1.2 cm, totally submitted.

Microscopic :

Sections examined from the specimen [illegible] groups of malignant epithelial cells showing [illegible] hyperchromasia, arranged in distorted acini and solid [illegible] desmoplasia ; No necrosis.

Diagnosis :

Left retropectoral mass, US guided tru cut biopsy, POORLY DIFFERENTIATED ADENOCARCINOMA.
- CASE FOR IMMUNOPHENOTYPING
- PLEASE CORRELATE WITH PREVIOUS PATHOLOGY

Prof. Dr. Shady Ella Anis

Prof. Dr. Ella Anis Ishak

[illegible]